

RUN DATE:
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PATIENT:

ACCT #:

REG DR:

AGE/SX:

ROOM

DOB:

BED:

STATUS:

SPEC #:

RECD:

STATUS:

COLL:

IME IN FORMALIN: 29:21 hrs.

CLINICAL INFORMATION:

Pre-Op Diagnosis: Breast CA

Remarks:

Specimen(s): Right breast stitch marks axillary contents

MICROSCOPIC DIAGNOSIS

BREAST, RIGHT, MODIFIED RADICAL MASTECTOMY:

- MULTIFOCAL INVASIVE DUCTAL CARCINOMA - SEE COMMENT
- LARGEST TUMOR MEASURES 4 CM; GRADE 2
- ANGIOLYMPHATIC INVASION PRESENT
- SEVEN (7) OF EIGHTEEN (18) AXILLARY LYMPH NODES POSITIVE FOR METASTATIC CARCINOMA
- WITH FOCAL EXTRACAPSULARY EXTENSION PRESENT
- MARGINS NEGATIVE

UUID:5C3D9179-7434-4776-A2FF-68CEA0149C68
TCGA-LL-A6FQ-01A-PR

Redacted



COMMENT(S)

In addition to the two larger tumors present within the upper outer quadrant, a tumor is also present associated with the nipple measuring 2 cm. This section was evaluated with E-cadherin with appropriate controls demonstrating that the infiltrating carcinoma is strongly positive for a E-cadherin confirming ductal type. Additionally, a couple of the lymph nodes were evaluated with PanKeratin stain with appropriate controls. A section from the larger mass was evaluated for a breast prognostic panel.

PROTOCOL FOR EXAMINATION OF SPECIMENS WITH INVASIVE CARCINOMA OF THE BREAST

PROCEDURE:

Total mastectomy

LYMPH NODE SAMPLING:

Axillary dissection

SPECIMEN LATERALITY:

Right

HISTOLOGIC TYPE OF

INVASIVE CARCINOMA:

Invasive ductal carcinoma (NOS)

TUMOR SIZE:

Greatest dimension of largest focus: 4 cm

HISTOLOGIC GRADE:

Nottingham histologic score

Glandular differentiation: Score 3

Nuclear pleomorphism: Score 2

Mitotic index: Score 1

Overall grade: Grade 2

Multiple foci of invasive carcinoma

No DCIS is present

TUMOR FOCALITY:

DUCTAL CARCINOMA IN SITU:

MACROSCOPIC AND MICROSCOPIC

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Continued)

COMMENT(S)

(Continued)

EXTENT OF TUMOR:

Skin: Invasive carcinoma directly invades into the dermis without skin ulceration

MARGINS:

Margins uninvolved by invasive carcinoma

Distance from closest margin: 3 cm

LYMPH NODES:

Number of sentinel lymph nodes examined: 0

Total number of lymph nodes examined (sentinel and non-sentinel): 18

Number of lymph nodes with macrometastasis: 5

Number of lymph nodes with micrometastasis: 2

PATHOLOGIC STAGING:

Primary tumor: pT2

Regional lymph nodes: pN2a

Distance metastasis: Not applicable

ANCILLARY STUDIES:

See microscopic description (ER+, PR+, Her2-)

GROSS DESCRIPTION:

Received fresh for selection of tissue for tissue banking labeled with the patient's name is a modified radical mastectomy specimen. The axillary contents measure 16 x 11 x 4 cm, palpation of which reveals clinically suspicious lymph nodes. The breast weighs 1.457 g and measures 30 x 18 x 6 cm. Skeletal muscle fibers are present on the deep margin. A large mass is palpated in the upper outer quadrant. The deep margin adjacent to this area is inked blue. The breast is covered by a 30 x 18 cm ellipse of dark brown skin. The nipple and areolar complex is centrally placed. The nipple measures 2.5 cm in diameter and is somewhat firm on palpation. The breast is serially sectioned revealing a well-circumscribed grayish-white tumor within the upper outer quadrant measuring 3 x 3.5 x 4 cm. A portion of the tumor is provided to the tissue bank coordinator. The deep margin measures 3 cm. The tumor lies 1.5 cm from the skin surface. On further evaluation, there is an apparent separate tumor in the upper outer quadrant with a core needle biopsy track and this tumor measures 1.5 x 1 x 1 cm. This tumor is well away from the surgical margins. The remainder of the breast tissue is fatty in nature. No other discrete lesions are identified. Examination of the axillary contents reveals multiple lymph nodes which range from 0.5 cm to 3.5 cm in greatest dimension each. Some of these are grossly involved by tumor. Some are fatty in nature.

Cassette Summary:

- 1 - nipple
- 2 - deep margin adjacent to larger tumor
- 3-7 - sections of larger tumor
- 8 - tissue between larger tumor and smaller tumor
- 9,10 - sections of smaller tumor
- 11-13 - sections of lateral, central, and medial breast tissue, respectively
- 14 - two highest axillary lymph nodes
- 15 - smaller whole lymph nodes
- 16-21 - one lymph node each sectioned and entirely submitted
- 22,23 - representative section of two large fatty lymph nodes
- 24-26 - representative section of each grossly positive lymph node

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MICROSCOPIC DESCRIPTION:
BREAST PROGNOSTIC PANEL - LARGEST TUMOR

ASSAY TYPE	% TUMOR STAINED	INTENSITY	RESULTS
Estrogen Receptor	100	3	POSITIVE
Progesterone Receptor	85	2	POSITIVE
Ki-67	24	2	HIGH
Her2/neu	SCORE 1+		NEGATIVE

REFERENCE RANGES: ER/PR: Negative <1%, Positive >=1%
: Ki-67: Low <10%, Borderline 10-20%, High >20%
: Her2/neu: Negative 0-1+, Equivocal 2+, Positive 3+
: Her2/neu results are reflexed to FISH.
All immunohistochemical staining and the were performed at
performed scoring and

interpretation utilizing

Antibody clones: Her2/neu, Clone 6F11; Clone PgP1294; Ki-67, Clone MIB-1

CAP/ASCO guidelines for ER and PgR IHC: formalin fixation time 6-72 hours; cold ischemia time 1 hour or less. CAP/ASCO guidelines for Her2 testing: formalin fixation time 6-48 hours; cold ischemia time 1 hour or less. Testing outside of these guidelines may affect results.

INTRAOPERATIVE CONSULTATION:

RIGHT BREAST TISSUE:

- TUMOR ADEQUATE FOR TISSUE BANKING WITH PORTION OF TUMOR PROVIDED TO TISSUE BANK COORDINATOR

Image
Image
Image
Image

Signed ____ (signature on file) ____

** END OF REPORT **

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
HPVAA Discrepancy		✓
Prior Malignancy History		✓
Dual/Synchronous Primary Noted		✓
Care in Facility:	QUALIFIED	DISQUALIFIED
Reviewed by:	LMW	5/23/13